Abstract citation ID: bvae163.270

Adrenal (Excluding Mineralocorticoids)

6794

Primary Adrenal Insufficiency Following Bilateral Adrenal Hemorrhage Secondary To Antiphospholipid Syndrome

Majid Alameri, MD[1], Abdulla Mohamed Alnuaimi, MBBS[2], Yousef Alabrach[3], Aymen Abbas[3], Sarah Baroud[4], and Mohammed El Khazendar[4]

[1]SKMC Center for Diabetes & Endocrinology, Abu Dhabi, United Arab Emirates; [2]SKMC Center for Diabetes & Endocrinology, Abu Dhabi, United Arab Emirates, Abu Dhabi, United Arab Emirates; [3]Department of Internal Medicine, Sheikh Khalifa Medical City (SKMC), Abu Dhabi., Abu Dhabi, United Arab Emirates; [4]Department of Internal Medicine, Sheikh Khalifa Medical City (SKMC), Abu Dhabi, Abu Dhabi, United Arab Emirates

Disclosure: M. Alameri: None. **A.M. Alnuaimi:** None. **Y. Alabrach:** None. **A. Abbas:** None. **S. Baroud:** None. **M. El Khazendar:** None.

Introduction: Adrenal insufficiency (AI) is a rare life-threatening complication of antiphospholipid antibody syndrome (APS) accounting for 0.4% to 0.5% of AI cases. **Case report:** A 26-year-old female with background history of primary antiphospholipid syndrome (PAPS) presented with 3 weeks history of dull abdominal pain and fatigue. Examination revealed dehydration, orthostatic hypotension and hyperpigmented skin. Biochemistry assessment was consistent with primary adrenal insufficiency (hyponatremia 125 mol/L, hyperkalaemia 6.8 mmol/L, low random cortisol 142 mol/L, high ATCH 440 pmol/L [N 1.6-13.9], low aldosterone <1.05 ng/dL and high renin 128 ng/L [N 4-23.7]). She received 100mg hydrocortisone followed by regular IV hydrocortisone q6hrs. CT abdomen revealed bilaterally enlarged hyperdense adrenal glands suggestive of haemorrhage. Adrenal MRI confirmed subacute bilateral adrenal haemorrhage (BAH) replacing whole adrenal glands with hemosiderin depositions and calcifications without evidence of underlying tumor. Anti-21-hydroxylase antibody came back negative. She was discharged on hydrocortisone, fludrocortisone and hydroxychloroquine. Education was given for sick days rule and hydrocortisone emergency kit was provided for adrenal crisis. **Discussion and conclusion:** We describe a patient with APS, who developed AI due to bilateral subacute adrenal haemorrhages. Adrenal haemorrhage is rare but life-threatening complication of APS. High clinical vigilance is important to consider adrenal haemorrhage in differential diagnosis of AI, specifically in patients with known antiphospholipid antibody syndrome (APS).

Presentation: 6/3/2024